INSTITUTIONAL EQUITY RESEARCH · REFRESH / RE-UNDERWRITE

# UnitedHealth Group (UNH)

Q3 2026 Post-Earnings Update — Recovery Strengthens, but Clean Underwriting Proof Still Matters

Report date: October 17, 2026  Reference price: $401.43 (Oct. 16 close)

Portfolio source: retirement-portfolio-63.xlsx

## Investment view: ADD · Conviction 7/10

**Bottom line.** Q3 materially strengthened the turnaround case: adjusted EPS beat expectations, the 2026 adjusted-EPS floor rose to at least $20.50, UnitedHealthcare margins improved, and management now frames the low end of its 13%–16% long-term growth range as a reasonable 2027 starting point. The stock has already rerated from the immediate post-print lows, however, and the quarter still included $610M of favorable reserve development. At $401.43, UNH remains attractive enough to add in stages, but no longer offers the same asymmetry it did in the mid-$370s.

**12–18 month base value: $465**, implying roughly 16% price upside before the dividend. Bull $530; bear $320.

| RECOMMENDATION | CONVICTION |
|---|---|
| ADD | 7 / 10 |
| **BASE VALUE** | **UPSIDE** |
| $465 | +15.8% |

**DECISION DISCIPLINE**

Prefer staged adds below ~$410; become materially more aggressive below ~$385 if the thesis remains intact. Above ~$455, require stronger 2027 evidence.

| Q3 REVENUE | ADJUSTED EPS | MEDICAL CARE RATIO | OPERATING INCOME | FY26 ADJ. EPS |
|---|---|---|---|---|
| $112.7B | $4.69 | 88.4% | $6.6B | ≥$20.50 |
| vs. $113.2B y/y | vs. $2.92 y/y | 89.9% y/y | $4.3B y/y | raised outlook |

**Contents:**

What changed  Earnings  Earnings quality  Business quality  Valuation  Portfolio fit  Scenarios  Checkpoints  Risks  Sources

# 1. What has changed since the previous underwriting?

IMPROVED

## The earnings recovery has more evidence

Q3 adjusted EPS was $4.69, operating income rose to $6.6B from $4.3B a year earlier, and UnitedHealthcare operating margin improved to 3.3% from 2.1%. Management lifted 2026 adjusted EPS to at least $20.50.

IMPROVED

## 2027 growth framing is stronger

CFO Wayne DeVeydt said the low end of UNH's long-term 13%–16% growth range is a more reasonable 2027 starting point than current consensus, implying roughly $23.17+ of adjusted EPS from the 2026 floor.

STILL UNRESOLVED

## Medical-cost normalization is not yet clean

The 88.4% medical-care ratio improved year over year, but benefited from $610M of favorable development on earlier 2026 service dates. Commercial medical trend is still running above 11%.

MORE DEMANDING

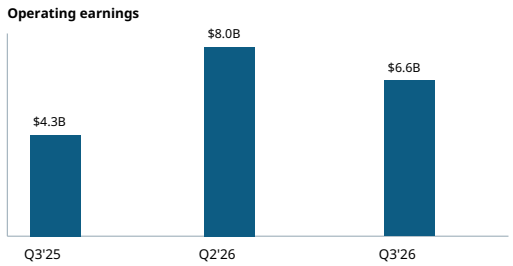
## Valuation has rerated

UNH closed at $401.43 on Oct. 16 versus roughly $376 immediately after the print. That removes part of the valuation cushion: price/2026 adjusted EPS floor is now ~19.6x.

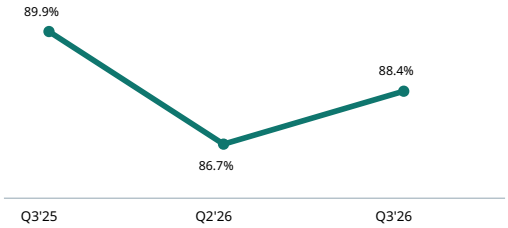
> **Net assessment:** The thesis is stronger, but the security is modestly less attractive after the post-earnings rebound. The right response is to retain an **Add** rating, not upgrade to Strong Add.

# 2. Q3 earnings and operating trajectory

## Quarterly operating trend




## Medical care ratio




| Metric | Q3 2025 | Q2 2026 | Q3 2026 | Read-through |
|---|---|---|---|---|
| Revenue | $113.2B | $112.0B | $112.7B | Top line remains subdued as membership/product mix is reset. |
| Operating earnings | $4.3B | $8.0B | $6.6B | **Recovery is real, though Q2/Q3 seasonality and reserve effects matter.** |
| Adjusted EPS | $2.92 | $6.38 | $4.69 | **Substantial y/y improvement; full-year floor raised.** |

| Metric | Q3 2025 | Q2 2026 | Q3 2026 | Read-through |
|---|---|---|---|---|
| Medical care ratio | 89.9% | 86.7% | 88.4% | Better y/y, but still elevated and aided by reserve development. |
| Operating cash flow | — | $11.1B | $5.1B | 1.2x Q3 net income; adequate, but monitor full-year conversion. |

### Segment evidence

UnitedHealthcare produced $86.3B of revenue and $2.8B of operating earnings, with 3.3% operating margin versus 2.1% a year earlier. Optum generated $66.9B of revenue and $3.7B of operating earnings. Optum Health's operating performance has improved sharply from the prior-year trough, while Optum Insight remains an investment/remediation story and Optum Rx is emphasizing a simpler, more transparent commercial model.

## 3. Earnings quality: the key reason conviction stays at 7/10

> **EPS quality screen triggered.** Q3's 88.4% medical-care ratio included **$610M of favorable reserve development** tied to 2026 dates of service. That is economically supportive—it indicates earlier estimates were conservative—but it also means headline margin improvement should not be extrapolated one-for-one.

| Bridge item | Approx. effect | Interpretation |
|---|---|---|
| Reported adjusted EPS | $4.69 | Strong reported quarter. |
| Favorable reserve development | $610M pre-tax | Supports claims adequacy, but lifts reported underwriting economics. |
| Illustrative after-tax EPS equivalent | ~$0.53 | Uses ~22% tax and ~900M shares; analytical estimate, not company guidance. |
| Reserve-adjusted lens | ~$4.16 | Still respectable, but materially less spectacular than headline EPS. |

The reserve-adjusted EPS illustration is deliberately conservative and should not be read as a restatement. Reserve development is a normal insurer accounting feature; the issue is whether future underwriting improvement persists without recurring large favorable releases.

## 4. Business quality, management and capital allocation

### Company Quality: High / repair phase

UNH's scale, Optum breadth, distribution, data assets, purchasing power and integrated care capabilities remain strategic advantages. The 2025–26 disruption damaged trust and operating consistency, but the underlying franchise still has unusually broad healthcare touchpoints.

### Management credibility: improving

Stephen Hemsley's return has coincided with a sharper focus on operational discipline, simplified processes, clinician relationships and accountable business-level execution. Management is giving more concrete productivity and modernization markers.

### Capital allocation: balanced

Debt-to-capital was 41.7%; management still targets ~40% by year-end. UNH repurchased $5B of stock and paid $6.2B of dividends through Q3, while continuing technology and care-delivery investment.

## AI and modernization are becoming measurable rather than purely narrative

Management said AI-enabled chart summarization is deployed to more than 40% of employed clinicians and has reduced chart review time by 33%; AI-enabled information gathering shortened appeal cycle times by 26%. These are credible operating-productivity proof points, but the valuation case should not capitalize them aggressively until they show up in clean margin and cash-flow progression.

## Medicare Advantage strategy: quality over raw membership

UnitedHealthcare continues to prioritize high-value HMO and special-needs products and is deliberately accepting membership attrition where benefit economics are unattractive. The Q3 materials show 48.3M total UnitedHealthcare consumers, while recent disclosures indicate roughly 935,000 fewer Medicare Advantage members since year-end 2025. The key question is whether 2027 pricing, Stars and risk mix rebuild profit faster than membership shrinks.

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# 5. Valuation and market expectations

| Measure | UNH |
|---|---|
| Oct. 16 close | $401.43 |
| 2026 adjusted EPS floor | $20.50 |
| Price / 2026 EPS floor | 19.6x |
| Provider forward P/E | 17.7x |
| PEG (provider) | 1.25x |
| Consensus target | $477.04 |
| Dividend yield | ~2.3% |

| Peer | Price | Forward P/E |
|---|---|---|
| UnitedHealth | $401.43 | 17.7x |
| Elevance Health | $428.26 | 15.8x |
| CVS Health | $92.36 | 11.4x |
| Humana | $413.16 | 33.3x |

> **Valuation Attractiveness: Attractive, not distressed.** UNH now trades at a premium to ELV and CVS, which is reasonable only if integrated Optum economics and 2027 EPS recovery prove superior. The stock is still far below the premium multiple historically accorded to a clean, compounding UNH, but the burden of proof is also materially higher than it used to be.

## 2027 earnings frame

Applying management's low-end 13% growth framing to the $20.50 2026 adjusted-EPS floor yields about **$23.17** of 2027 EPS; 16% yields about **$23.78**. At today's price that is roughly **16.9x–17.3x**. A 19.5x multiple on ~$23.8 supports a value near $464, which anchors the base-case target.

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# 6. Portfolio Fit — retirement-portfolio-63.xlsx

| Holding detail | Value |
|---|---|
| Shares | 50.22 |
| Workbook last price | $402.27 |
| Workbook market value | $20,191.96 |
| Portfolio weight | 1.17% |
| Total cost basis | $11,924.74 |
| Average cost | $237.45 |

| Holding detail | Value |
|---|---|
| Workbook fair value annotation | $475 |

> **Portfolio Fit: Favorable.** At ~1.17% of portfolio value, UNH is large enough to matter but not large enough to create concentration risk. It also diversifies a portfolio with substantial technology, financial and industrial exposure. The strongest portfolio argument for adding is not merely expected return; it is the combination of defensive healthcare demand, improving company-specific earnings and modest current weight.

**Sizing framework:** maintain a path toward roughly 1.5%–1.75% only if reserve-adjusted MCR improvement persists and the formal 2027 outlook is at least consistent with ~$23+ adjusted EPS. Avoid pushing above ~2% before regulatory and claims-quality proof improves.

## 7. Bull / Base / Bear valuation

### Bear — $320

**Probability: 25%**

2027 EPS near $21–$22; MCR remains near 89%–90%; Optum margin recovery stalls; regulatory or funding pressure keeps the multiple near 14x–15x.

### Base — $465

**Probability: 55%**

2027 adjusted EPS ~$23.5–$24.0, reserve-adjusted medical costs improve, Optum margins progress and valuation normalizes to ~19.5x.

### Bull — $530

**Probability: 20%**

EPS reaches ~$24.5+, 2027 MA economics surprise positively, Optum acceleration is broad, and the market restores a low-20s multiple.

**Probability-weighted value: ~$439.** This supports positive expected value but reinforces staged entry discipline after the recent rebound.

## 8. Catalysts, checkpoints and add gates

| Checkpoint | What would be positive | What would weaken the thesis |
|---|---|---|
| Formal 2027 outlook | Adjusted EPS ≥ ~$23.2 with credible underlying margin bridge. | Guide below ~$22 without a temporary, well-explained driver. |
| Medical care ratio | Reserve-adjusted MCR trends lower with commercial trend stabilizing. | MCR ~89%+ despite repricing, especially with adverse reserve development. |
| Medicare Advantage AEP | Profitable mix holds up; HMO/SNP growth offsets planned PPO exits. | Membership loss materially exceeds economics benefit or Stars deteriorate. |
| Optum Health | 3%+ margin holds and progresses toward long-range goals. | Margin relapses toward 0%–1% or VBC contracts remain uneconomic. |
| Cash / leverage | Debt-to-capital approaches 40% with clean cash conversion. | Leverage rises while buybacks continue ahead of remediation. |

### Strongest add gate

**<$385** with thesis intact: valuation again offers a meaningful buffer even before full 2027 proof.

### Normal add zone

**$385–$415**: use staged purchases; current price falls in this range.

### Wait-for-proof zone

**>$455**: expected return compresses; require cleaner claims and 2027 confirmation.

## 9. Risks, contrary evidence and thesis-break conditions

### Most important contrary evidence

- Commercial medical trend remains above 11%, driven by No Surprises Act costs, clinician intensity and specialty drugs.
- Q3 MCR improvement was not fully organic because of $610M favorable reserve development.
- Medicare Advantage membership is lower by roughly 935,000 since year-end 2025.
- DOJ, CMS/RADV and PBM/regulatory scrutiny can create direct repayments, operating constraints or a permanently higher risk premium.
- Revenue is not currently growing; the near-term case depends more on margin repair than top-line expansion.

### Explicit thesis breaks

- MCR persists around or above 89% despite 2027 repricing, particularly with adverse reserve development.
- Credible 2027 adjusted EPS falls to ~$21 or below, or the 13% growth bridge is materially abandoned.
- Optum Health margin relapses toward 0%–1% or value-based-care economics deteriorate structurally.
- A major DOJ/CMS/PBM outcome causes material repayments, structural restrictions or durable margin erosion.
- Balance-sheet leverage worsens materially while capital returns remain aggressive.

> **Research conclusion:** Q3 reduces the probability of a failed turnaround, but it does not eliminate underwriting-quality and regulatory tail risk. That combination justifies **Add / 7 of 10**, not Strong Add.

## 10. Sources, methodology and limitations

- UnitedHealth Group, Q3 2026 earnings release, Oct. 13, 2026: company release.
- UnitedHealth Group, Q3 2026 prepared earnings remarks: prepared remarks.
- SEC-filed Q3 earnings materials: SEC exhibit.
- Reuters, Oct. 13, 2026, earnings and 2027 growth commentary: Reuters.
- CMS, 2027 Medicare Advantage & Part D Star Ratings, Oct. 8, 2026: CMS.
- StockAnalysis market/valuation and analyst-consensus pages, checked Oct. 17, 2026: UNH overview and analyst forecast.
- Peer market data: StockAnalysis pages for ELV, CVS and HUM, checked Oct. 17, 2026.
- Portfolio holdings, allocation and cost basis: **retirement-portfolio-63.xlsx**, treated as the sole portfolio source for this report.

**Source-control note:** the repository agent-state manifest still names retirement-portfolio-61.xlsx, which is stale relative to the user's explicit Oct. 16 designation of retirement-portfolio-63.xlsx. This report uses the uploaded version 63 workbook directly and does not substitute the older manifest workbook. Market prices and consensus data are refreshed separately from the workbook. Scenario values, add gates and reserve-adjusted EPS are analyst estimates, not company guidance.

Prepared for portfolio-management use. Recommendation scale: Strong Add / Add / Hold / Watch / Trim / Exit. This is investment research, not tax or legal advice.